

Market, competitors and pricing logic

For the STYL product team. Read this with the Sales Sheet. Do not send to customers.

Summary

What we sell One product: Ward. It locates staff and equipment to the bed (± 20 cm) and makes the ward reports. Two ways to host it.	Where we sell Singapore first: S\$12.9M a year in FY28 (public and private). Japan next, self-hosted only: about S\$249M a year.	Price and margin S\$67.50 per bed per month in bays (cloud). On the same floor area, at or below the closest UWB competitor. Gross margin about 80% at 500 beds.
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The offer

	Ward — Cloud hosted	Ward — Self-hosted
Software	S\$450 per bed per year	S\$450 per bed per year
Platform fee	Included	S\$95,000 per site per year
Servers and GPU	STYL provides	Hospital provides
Hardware service	S\$30 per bed per month in bays (3 or more beds). S\$60 in rooms with 1 or 2 beds. 1 tag per bed.	
Set-up	One-time fee after the site survey. Installation priced for each anchor.	
Markets	Singapore	Singapore and Japan

Serviceable market (SAM), FY28: August 2027 to July 2028

Market	Beds	SAM per year *
Singapore, acute: 2025 beds (public 10,784, not-for-profit 334, private 1,649), plus 600 public beds that open by 2027 (SKH, SGH, CGH)	13,367	S\$10.8M
Singapore, community hospitals: public and not-for-profit, 2025 beds	2,579	S\$2.1M
Japan: general beds in 735 hospitals with 400+ beds, public and private. 2023 count less 4%	306,900	S\$249M
Not counted: psychiatric beds (1,950; staff safety needs a help button); Alexandra Hospital (from 2028) and Eastern General (from 2029); Japan hospitals with 200–399 beds	—	—

* Conservative: S\$810 per bed per year (software S\$450 + hardware at the bay rate S\$360), before discount. No platform fee and no 1–2 bed room rate counted. Japan general beds fell 2.5% from March 2023 to April 2026, so we take off 4% for FY28. Japan price is not tested. Sources: MOH Singapore Health Facts 2025; MOH Committee of Supply 2025; MHLW Survey of Medical Institutions 2023 (table Z15); MHLW monthly hospital survey, March 2023 and April 2026.

Region and global: we add them only when Solace becomes our marketing arm. This needs a signed co-sell agreement, Atlantic listed as a Solace Agent Mesh reference solution, and one live hospital reference.

Where we win - Accurate to the bed (± 20 cm) in shared bays. Position 2 times each second. - Cloud hosted in Singapore, or self-hosted. - Operates alongside Wi-Fi, RFID and nurse call. Uses data from other location systems and sensors. - Ward reports and questions in plain language. - Local team, 18 years in Singapore, ISO 27001.	Where we are weak today - No hospital reference yet. - No help button on the badge. - Nurse call link needs the supplier's API. - No link to patient records yet. - Our tag needs a charge each week (test for 2 weeks in progress). It does not suit equipment.
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Room-level systems are common. Bed-level is not.

Singapore and Japan.

Competitors a Singapore hospital can choose

Group	Technology and accuracy	Strong point	Weak point	Our answer
Local incumbent Cadi Scientific	RFID and Wi-Fi. About 2–2.5 m.	In TTSH, SGH, SKH, KTPH and KKH. Local support.	Not accurate to the bed. One study: 72% of contacts found.	Add STYL where the hospital needs the bed.
Clinical UWB Baxter (Hillrom)	UWB. Less than 1 m.	Connects to its own nurse call and beds.	Hospital must use Baxter systems. Tag charge every 72 hours.	Any nurse call with an API. Badge charge lasts 1 week.
UWB service (UK) Proximos	UWB (Iiwari). ±25 cm, each 10 s.	Same accuracy. Price includes tags and anchors. Tag battery 2 years.	Cloud in UK and EEA only. Minimum 1,000 tags and 15 tags per 100 m².	Position 20 times faster. Price per bed, no area rule. Singapore cloud or self-hosted.
US hospital RTLS CenTrak, Midmark, Sonitor, Securitas	Infrared or ultrasound for the room. BLE at 3–5 m.	Large installed base. Many hospital system links.	Equipment in each room. No local team.	Accurate to the bed without equipment in each room.
Cloud and network Kontakt.io, Cisco, HPE	BLE and Wi-Fi. Room or zone.	Low cost. Uses the hospital Wi-Fi.	Cloud only. Not accurate to the bed.	Cloud or self-hosted. Atlantic can also use their data.
Industrial UWB Pozyx, Sewio, Ubisense	UWB. 10–30 cm.	Good accuracy. Public kit prices.	No hospital functions or references.	We sell ward results, not location points.

Threats to monitor

Threat	Why it is important	Our action
Cadi adds UWB through Cisco Wi-Fi 7	Cadi keeps its hospitals and closes our accuracy gap.	Make Atlantic accept Cisco and Cadi data.
Baxter sells UWB in Asia	Same accuracy, with a large brand.	Win the first hospital reference first.
Proximos or Iiwari enters Asia	Same UWB accuracy. Iiwari already sells nurse call routing and staff alarms.	Win the first references. Keep the price per bed.
Bluetooth Channel Sounding	Low-cost BLE gets to about 0.3–1 m from 2027.	Keep the message on the bed (±20 cm).

Japan: self-hosted only

- **Rule:** In Japan, we do not sell the cloud hosted option. Each hospital gets Atlantic in its own data centre.
- **Reason:** The MHLW security guideline for medical information systems requires equipment to be where Japanese law applies.
- **Radio:** MoonFish uses UWB channel 9 only. Japan permits channel 9. Each anchor and tag needs Japanese radio certification (技適).

Japan: what hospitals use today

Supplier	Technology	Use and references
Dr.JOY	BLE beacons, cloud	Doctor working time. 170+ facilities, 51 university hospitals.
Beacapp Here (also Fujitsu)	BLE beacons, cloud. Room level.	Attendance, nurse movement, equipment. ¥33,000–110,000 per month.
CARECOM, Panasonic	Nurse call, beacons in lights	Nurse movement studies, staff search. Possible partners.
UWB suppliers	UWB	No hospital in Japan found. Hitachi stopped selling Quuppa in March 2026.

Result: Room-level BLE is in use, mostly for doctor working time. Bed-level location, ward reports and self-hosted AI are not in use. This is a possible need, not an empty market.

Why Japanese hospitals can buy

- Doctor overtime limits (since April 2024) need objective records of working time.
- The 2026 fee revision lets wards with ICT staff 10% below the nurse ratio, with an annual workload check.
- An MHLW subsidy gives up to ¥80M for each hospital for ICT that makes work more efficient.

Why it is difficult

- About 60–74% of hospitals had a loss in FY2024.
- BLE costs much less: about ¥0.4–1.3M per year for one hospital.
- We need a healthcare partner: an EMR supplier (Fujitsu, NEC) or a nurse call supplier.

Why the price is simple and correct

Seven decisions. Each decision has a reason and a check.

Pricing decisions

Decision	Reason	Check
One product: Ward	Sales staff remember one product. Our value is staff location and ward reports. Equipment-only buyers choose low-cost BLE or RFID.	3D model and patient-record link are set-up options. We give up a higher-priced command-centre package until buyers ask for it.
Two ways to host	Customers choose. Each part of Atlantic can operate in the hospital, with no internet link. CAG already requires its own data centre.	Self-hosted pays a platform fee for the Solace licence and our support.
Price by bed, in three parts	Hospitals budget by bed. A price per tag puts us against BLE tags at US\$2–6.	Each bed includes 1 staff tag (1 staff to 1 bed). Extra tags: S\$800 per 10.
Unlimited users	Users add no cost. ThingsBoard CE is free. Solace counts only anchor connections.	AI questions are the only user cost. LiteLLM caps them in the cloud.
Two hardware prices: S\$30 in bays, S\$60 in 1–2 bed rooms	Each room needs at least 3 anchors, for any number of beds. A bay uses 0.5 anchors per bed. A 1–2 bed room uses 1.5–3.	Sales asks for the room types in the first meeting. The survey confirms them before the quote.
One list price, no volume table	The Sales Director can already give up to 20%. One price is easier to remember.	No quote below 30% margin (page 4).
Minimum fee: S\$60,000 for each site each year	Each cloud site has a fixed cost of S\$3,200 per month.	Small sites stay above the 30% floor (page 4).

Self-hosted platform fee: S\$95,000 per site per year

Cost line	S\$ per year
Solace licence for production and disaster recovery (at least US\$50,000)	65,000
Our support for the hospital's system: 0.15 person *	21,600
Cost. The fee adds about 10% for currency and price risk.	86,600

Price check: the closest competitor (Proxximos, UK)

List price, 3-year term: £14.90 per person tag per month, £7.90 per asset tag, £51.50 set-up for each tag. The set-up fee includes the anchors. Rules: at least 1,000 tags, and at least 15 tags for each 100 m². The density rule makes the hospital pay for the anchors by floor area, not by bed.

Basis (Proxximos: cheapest tag mix, set-up fee included)	Tags per bed	S\$ per bed per month
1 tag per bed (not permitted by their rules)	1.0	28
Dense ward with 6-bed bays, 25 m² per bed *	3.8	72
NHS ward, net area (29.5 m² per bed)	4.4	83
NHS ward, gross area (48.6 m² per bed)	7.3	129
STYL Ward, cloud: bays / rooms with 1–2 beds. Set-up is extra.	1.0	67.50 / 97.50

£1 = S\$1.72. Sources: Proxximos G-Cloud 15 price list and service definition (January 2026); NHS HBN 04-01 (24-bed ward).

What this means: on the same floor area, our list price is at or below theirs. They can give a bespoke price below their rules. Their tag runs 2 years without a charge. Our tag needs a charge each week, but gives a position 2 times each second (theirs: 1 each 10 s). Room-level BLE in Japan costs much less (Beacapp: about S\$2 per bed). Test the price with the first three hospitals.

The value is larger than the price

A 500-bed hospital has about 150 nurses on each shift, with 3 shifts each day. If each nurse saves 10 minutes each shift, the hospital gets back **about 27,000 nurse hours each year (about 15 full-time nurses)**. At S\$60,000 for each nurse each year *, this is about **S\$900,000 a year**. The cloud hosted price is S\$405,000 a year.

Design unit from the pilot

6-bed bay 3 anchors for line of sight. 0.5 anchors per bed.	1–2 bed room 3 anchors for each room. Hardware S\$60 per bed.	6 tags Included for each bay (1 per bed). The pilot used 10.	24 tags At one time in a bay (3 anchors × 8). Pilot peak: 4 nurses.
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The margin stays above 30% when the room types are correct

Margin = (revenue – cost) ÷ revenue. Our floor is 30%. * = assumption, to confirm.

Base cost: 500 beds in bays, cloud hosted, per month

Cost line	Basis	S\$
Cloud backend, Solace included	Fixed for each site	2,000
Platform support	0.1 person × S\$12,000 *, fixed for each site	1,200
Anchors: 250 × S\$100	Paid over 60 months	417
Tags: 500 × S\$40 (1 per bed)	Paid over 60 months	333
Spare parts, repairs, field service	10% of hardware each year *; S\$1.60 per bed *	1,175
AI questions: 15,000 × S\$0.10	Full allowance of 30 per bed	1,500
Total cost: S\$13.25 per bed. Revenue: S\$67.50 per bed. Gross margin 80%		6,625

Stress test: what if things go wrong?

What goes wrong (500 beds, cloud, unless stated)	Revenue per bed	Cost per bed	Margin
Nothing (base case)	67.50	13.25	80%
A. Anchors and tags cost 50% more	67.50	14.38	79%
B. All beds are in 1–2 bed rooms (3 anchors per bed, S\$60)	97.50	19.50	80%
C. AI questions cost two times more (S\$0.20)	67.50	16.25	76%
D. Maximum discount (20%)	54.00	13.25	75%
A + B + C + D together	78.00	26.75	66%
Sales error: 1–2 bed rooms sold at S\$30, with A + C + D	54.00	26.75	50%
Small site: 100 beds, A + B + C + D	98.00	54.75	44%
Very small site: 40 beds, A + B + C + D	173.00	108.75	37%
Sales error on a small site: 100 beds, 1–2 bed rooms sold at S\$30, A + C + D	74.00	54.75	26%
Self-hosted: 500 beds, A + B + D	93.83	28.78	69%
Self-hosted: 100 beds, A + B + D	177.17	88.92	50%

S\$ per bed per month. Small sites pay the S\$60,000 minimum fee. Self-hosted: the hospital's GPU answers the AI questions, so C does not apply.

What the stress test tells us

- **Large sites are safe.** At 500 beds, the margin stays at 50% or more, even when all four problems occur together.
- **The S\$60 rate removes the single-room risk.** More anchors in 1–2 bed rooms now bring more revenue.
- **The remaining risk is a sales error on a small cloud site:** 1–2 bed rooms sold at the bay rate, plus a discount. Each cloud site has a fixed cost of S\$3,200 per month (S\$32 per bed at 100 beds).
- **Rules:** The survey confirms room types before the quote. Each discount gets a margin check: no quote below 30%. With correct room types, each case stays at 36% or more, down to 40 beds, at the maximum discount. The minimum fee protects small sites.
- **Self-hosted is safe at all sizes.** The platform fee pays the fixed cost.

Pilot cost. Atlantic-in-a-box with Solace Community Edition (pilots and proof of concept only, not production), so no Solace licence. 40 beds, 90 days: 21 anchors, 70 tags and the box S\$6,100 + support S\$3,600 + survey and set-up S\$12,000 * = S\$21,700. The fee of S\$40,000 gives a margin of 46%. It works in Singapore and Japan.

Margin and discounts. No quote below 30% gross margin. Target 40% for each customer over 24 months. Sales Director: up to 20%. CEO: more than 20%. No discount on the minimum fee or on the platform fee.

Open items. 1. Measure the cost per AI question in the pilot (S\$0.10 assumed). 2. Test coexistence in the pilot: UWB, BLE, Wi-Fi and medical equipment. 3. Help button: add our location to the current alarms, or build a button. 4. Japan: 技適 plan, healthcare partner and price test. 5. Tag battery test (this week): target 2 weeks between charges. 6. Test the price with the first three hospitals.